

BIOSYNC CLINICAL LABORATORY REPORT — URGENT

Patient: Harold J. Kowalski (DOB: 11/30/1955) | Date: April 17, 2026

Vital Signs & Biometric Readings

Heart Rate	118 BPM
Blood Pressure	178/116 mmHg
SpO2	87%
Temperature	99.2°F
Glucose	312 mg/dL
Weight	208 lbs
BMI	29.8
HRV	17 ms

Clinical Notes

Patient brought in by wife Donna Kowalski via ambulance. Patient experienced acute chest pressure, severe shortness of breath, and diaphoresis at home. Contrast dye allergy documented — CT protocol adjusted. Currently on Metoprolol 50mg, Amlodipine 5mg, Atorvastatin 40mg, Metformin 1000mg, Omeprazole 20mg, Allopurinol 200mg, Furosemide 40mg, Aspirin 81mg. Heart rate critically elevated at 118 BPM — troponin ordered, EKG showing ST changes in leads V3-V5 concerning for NSTEMI. Blood pressure 178/116 mmHg — hypertensive crisis, IV nitroprusside initiated. SpO2 critically low at 87% — high-flow oxygen at 10L/min via non-rebreather mask. Glucose 312 mg/dL critically elevated — insulin drip initiated. Weight 208 lbs — significant fluid overload. HRV 17 ms — severely depressed consistent with acute cardiac event. Cardiology emergency response team activated.

Provider Notes

NSTEMI suspected — cardiac catheterization lab on standby. Troponin I elevated at 2.8 ng/mL. Aspirin 325mg load given. Heparin drip initiated. IV Metoprolol administered. Contrast allergy protocol: premedicate with methylprednisolone and diphenhydramine before cath. ICU admission confirmed. Cardiology, endocrinology, and nephrology all consulted. Furosemide IV 80mg bolus for acute pulmonary edema. Wife updated on critical status — family at bedside. Patient intubated for airway protection. Condition critical. Ordering Provider: Dr. Thomas Brewer, MD — URGENT. Facility: BioSync Clinical Health Center — Cardiac ICU